

EMERGENCY DEPARTMENT WOMEN AND CHILDREN HOSPITAL KUALA LUMPUR

Severe Sepsis and Septic Shock Order Set

DATE :	TIME SEEN:
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PATIENT DETAILS

NAME		ADDRESS	
IC/PASSPORT		GENDER	MALE/FEMALE
AGE		RACE	MALAY/CHINESE/INDIAN/OTHERS _____
MRN		NEXT OF KIN CONTACT NO	

Weight (kg)		Allergy History		Co-morbid	
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DR. Signature/Time	ORDERS	Noted by SN Signature/Time
	PATIENT CARE Monitoring and access (includes all of the following orders): ☐ Blood pressure: _____ mmHg; HR: _____ x/min } Every 5 mins/ ☐ SPO2: _____ % under _____; Temperature: _____ c } until symptoms improve ☐ Continuous Cardiac Monitoring ☐ Place patient on supplemental oxygen (if required) ☐ Obtain 2 IV access (Notify MO stat if no assess within 5 minutes or 2 failed IV attempts)	
	LABORATORY Sepsis Panel ☐ FBC ☐ RP ☐ LFT/AST ☐ Blood C&S ☐ Dxt ☐ Lactate ☐ VBG ☐ Coagulation Profile Others (if indicated) ☐ GSH ☐ Urine C&S	
	FLUIDS Fluid Boluses (should be initiated within 15 minutes of arrival) Administer rapidly via IV push Reassess perfusion after every bolus to determine need for additional fluid Alternative fluids include Hartmann's/Ringer Lactate ☐ Bolus #1 : 0.9% NaCl _____ mL IV (20mL/kg, Max 40mL/kg) @ _____ (time) ☐ Bolus #2 : 0.9% NaCl _____ mL IV (20mL/kg) @ _____ (time) Consider starting IONOTROPES/VASOPRESSOR if persistent shock despite 40mL/kg of fluid Can repeat boluses if necessary (if rapid ongoing losses :AGE) ☐ Bolus #3 : 0.9% NaCl _____ mL IV (20mL/kg, Max 40mL/kg) @ _____ (time) ☐ Bolus #4 : 0.9% NaCl _____ mL IV (20mL/kg) @ _____ (time)	
	REASSESS ☐ Airway ☐ Breathing ☐ Circulation REFFERAL Team : Time of Referral : Spoken to : Team : Time of Referral : Spoken to :	

DR. Signature/Time	ORDERS	Noted by SN Signature/Time																								
	IONOTROPES/VASOPRESSOR <table border="1"> <tr> <td></td> <td>☐ WARM Shock</td> <td>☐ COLD Shock</td> </tr> <tr> <td>Peripheries</td> <td>☐ Warm, flushed</td> <td>☐ Cold, clammy, cyanotic</td> </tr> <tr> <td>CRT</td> <td>☐ <2s</td> <td>☐ >2s</td> </tr> <tr> <td>Pulse</td> <td>☐ Bounding</td> <td>☐ Weak, feeble</td> </tr> <tr> <td>HR</td> <td>☐ Tachycardia</td> <td>☐ Tachy/Bradycardia</td> </tr> <tr> <td>BP</td> <td>☐ Relatively maintained</td> <td>☐ Hypotension</td> </tr> <tr> <td>Pulse Pressure</td> <td>☐ Widened</td> <td>☐ Narrowed</td> </tr> <tr> <td>Choice of Ionotrope/Vasopressor</td> <td>NORADRENALINE</td> <td>ADRENALINE</td> </tr> </table> IVI Noradrenaline (CVC) ☐ 0.02-0.2mcg/kg/min put 0.06mg/kg in 50mL d5% at 1-10mls/hr ☐ 0.1-0.5mcg/kg/min put 0.15mg/kg in 50mL d5% at 2-10mls/hr IVI Noradrenaline (Peripheral) ☐ 0.05-0.1mcg/kg/min put 0.15mg/kg in 500mL at 10-20mL/hr IVI Adrenaline ☐ 0.15mg/kg in 50mL at 1-10mL/hr (0.05 -0.5mcg/kg/min) Dopamine (Alternative to Noradrenaline in highly selected patients eg: low risk of tachyarrhythmia, absolute or relative bradycardia, or if noradrenaline not available) Dobutamine is useful in patients with evidence of persistent hypoperfusion despite adequate intravascular volume		☐ WARM Shock	☐ COLD Shock	Peripheries	☐ Warm, flushed	☐ Cold, clammy, cyanotic	CRT	☐ <2s	☐ >2s	Pulse	☐ Bounding	☐ Weak, feeble	HR	☐ Tachycardia	☐ Tachy/Bradycardia	BP	☐ Relatively maintained	☐ Hypotension	Pulse Pressure	☐ Widened	☐ Narrowed	Choice of Ionotrope/Vasopressor	NORADRENALINE	ADRENALINE	
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	ANTIBIOTICS (Administer within 60 minutes) <3 mths : IV Cefotaxime 50mg/kg and C-Penicilin 50000u/kg IV Cefotaxime ____ mg and C-Penicillin ____ u >3 mths : IV Cefotaxime 50mg/kg or IV Ceftriaxone 50mg/kg IV Cefotaxime ____ mg or IV Ceftriaxone ____mg Other antibiotics (Administer within 60 minutes): - _____ - _____																									
Disposition: Admit to: _____ Spoken to: _____		Remarks:																								

ATTENDING DOCTOR
SIGNATURE AND STAMP

ATTENDING SN/MA
SIGNATURE AND STAMP